1. NAME OF THE DRUG PRODUCT

DIASIT 25
Sitagliptin Film Coated Tablets 25 mg

DIASIT 50
Sitagliptin Film Coated Tablets 50 mg

DIASIT 100
Sitagliptin Film Coated Tablets 100 mg

PHARMACEUTICAL DOSAGE FORM

Film-Coated Tablets

2. QUALITATIVE AND QUANTITATIVE COMPOSITIONS

Sitagliptin Tablets 25 mg
Each film-coated tablet contains
Sitagliptin hydrochloride monohydrate 28.343 mg
Equivalent to Sitagliptin … 25 mg

Sitagliptin Tablets 50 mg
Each film-coated tablet contains
Sitagliptin hydrochloride monohydrate 56.685 mg
Equivalent to Sitagliptin … 50 mg

Sitagliptin Tablets 100 mg
Each film-coated tablet contains
Sitagliptin hydrochloride monohydrate 113.370 mg
Equivalent to Sitagliptin … 100 mg

3. PHARMACEUTICAL FORM

Sitagliptin Tablets 25 mg
Pink colored, round shaped, biconvex, film-coated tablets, debossed with 'SG' on one side and '25' on the other side.

Sitagliptin Tablets 50 mg
Light Beige colored, round shaped, biconvex, film-coated tablets, debossed with 'SG' on one side and '50' on the other side.

Sitagliptin Tablets 100 mg
Beige colored, round shaped, biconvex, film-coated tablets, debossed with 'SG' on one side and '100' on the other side.

4. CLINICAL PARTICULARS

4.1 Therapeutic Indications

Monotherapy
DIASIT is indicated as an adjunct to diet and exercise to improve glycemic control in patients with type 2 diabetes mellitus.

Combination with metformin
DIASIT is indicated in patients with type 2 diabetes mellitus to improve glycemic control in combination with metformin as initial therapy or when the single agent alone, with diet and exercise, does not provide adequate

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glycemic control. Initial combination therapy or maintenance of combination therapy may not be appropriate for all patients. These management options are left to the discretion of the health care provider.

Combination with a sulphonylurea
DIASIT s indicated in patients with type 2 diabetes mellitus to improve glycemic control in combination with a sulphonylureas when treatment with maximal tolerated dose of sulphonylurea alone, with diet and exercise, does not provide adequate glycemic control and when metformin is inappropriate due to contraindications or intolerance.

Combination with metformin and a sulphonylurea
DIASIT is indicated in patients with type 2 diabetes mellitus to improve glycemic control in combination with metformin and a sulphonylurea when dual therapy with these two agents and with diet and exercise does not provide adequate glycemic control.

Combination with a peroxisome proliferator-activated receptor gamma (PPAR γ) agonist
DIASIT is indicated in patients with type 2 diabetes mellitus to improve glycemic control in combination with a PPAR γ agonist (i.e., thiazolidinediones) when diet and exercise, plus the single agent do not provide adequate glycemic control.

Combination with metformin and a PPAR γ agonist
DIASIT is indicated in patients with type 2 diabetes mellitus to improve glycemic control in combination with metformin and a PPAR γ agonist (i.e., thiazolidinediones) when dual therapy with these agents, with diet and exercise, does not provide adequate glycemic control.

Combination with Insulin
DIASIT is indicated in patients with type 2 diabetes mellitus as an adjunct to diet and exercise to improve glycemic control in combination with insulin (with or without metformin).

4.2 Posology and Method of Administration

The recommended dose of DIASIT is 100 mg once daily as monotherapy or as combination therapy with metformin, a sulfonylurea, insulin (with or without metformin), a PPAR γ agonist (i.e., thiazolidinediones), metformin plus a sulfonylurea, or metformin plus a PPAR γ agonist. DIASIT can be taken with or without food.

When DIASIT is used in combination with a sulfonylurea or with insulin, a lower dose of sulfonylurea or insulin may be considered to reduce the risk of sulfonylurea- or insulin-induced hypoglycaemia.

Patients with Renal Impairment

Because there is a dosage adjustment based upon renal function, assessment of renal function is recommended prior to initiation of DIASIT and periodically thereafter.

For patients with mild renal impairment (estimated glomerular filtration rate [eGFR] ≥60 mL/min/1.73 m^2 to <90 mL/min/1.73m^2), no dosage adjustment for DIASIT is required.

For patients with moderate renal impairment (eGFR ≥45 mL/min/1.73 m^2 to <60 mL/min/1.73m^2), no dosage adjustment for DIASIT is required.

For patients with moderate renal impairment (eGFR ≥30 mL/min/1.73 m^2 to <45 mL/min/1.73m^2), the dose of DIASIT is 50 mg once daily.

For patients with severe renal impairment (eGFR ≥15 mL/min/1.73 m^2 to <30 mL/min/1.73m^2) or with end-stage renal disease (ESRD) (eGFR < 15 mL/min/1.73 m^2), including those requiring haemodialysis or peritoneal dialysis, the dose of DIASIT is 25 mg once daily. DIASIT may be administered without regard to the timing of dialysis.

Paediatric population

DIASIT should not be used in children and adolescents 10 to 17 years of age because of insufficient efficacy. DIASIT has not been studied in paediatric patients under 10 years of age.

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Method of Administration
DIASIT can be taken with or without food.

4.3 CONTRAINDICATIONS

DIASIT is contraindicated in patients who are hypersensitive to any components of this product.

4.4 SPECIAL WARNINGS AND PRECAUTIONS FOR USE

General
DIASIT should not be used in patients with type 1 diabetes or for the treatment of diabetic ketoacidosis.

Acute Pancreatitis
Use of DPP-4 inhibitors has been associated with a risk of developing acute pancreatitis. Patients should be informed of the characteristic symptom of acute pancreatitis: persistent, severe abdominal pain. Resolution of pancreatitis has been observed after discontinuation of sitagliptin (with or without supportive treatment), but very rare cases of necrotising or haemorrhagic pancreatitis and/or death have been reported. If pancreatitis is suspected, DIASIT and other potentially suspect medicinal products should be discontinued; if acute pancreatitis is confirmed, DIASIT should not be restarted. Caution should be exercised in patients with a history of pancreatitis.

Hypoglycaemia when used in combination with other anti-hyperglycaemic medicinal products
DIASIT as monotherapy and as part of combination therapy with medicinal products not known to cause hypoglycaemia (i.e., metformin and/or a PPARγ agonist). Hypoglycaemia has been observed when sitagliptin was used in combination with insulin or a sulphonylurea. Therefore, to reduce the risk of hypoglycaemia, a lower dose of sulphonylurea or insulin may be considered.

Renal impairment
Sitagliptin is renally excreted. To achieve plasma concentrations of sitagliptin similar to those in patients with normal renal function, lower dosages are recommended in patients with GFR < 45 mL/min, as well as in ESRD patients requiring haemodialysis or peritoneal dialysis.

When considering the use of sitagliptin in combination with another anti-diabetic medicinal product, its conditions for use in patients with renal impairment should be checked.

Hypersensitivity reactions
Post-marketing reports of serious hypersensitivity reactions in patients treated with sitagliptin have been reported. These reactions include anaphylaxis, angioedema, and exfoliative skin conditions including Stevens-Johnson syndrome. Onset of these reactions occurred within the first 3 months after initiation of treatment, with some reports occurring after the first dose. If a hypersensitivity reaction is suspected, DIASIT should be discontinued. Other potential causes for the event should be assessed, and alternative treatment for diabetes initiated.

Bullous pemphigoid
There have been post-marketing reports of bullous pemphigoid in patients taking DPP-4 inhibitors including sitagliptin. If bullous pemphigoid is suspected, DIASIT should be discontinued.

Sodium
This medicinal product contains less than 1 mmol sodium (23 mg) per tablet, that is to say essentially 'sodium free'.

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4.5 INTERACTION WITH OTHER MEDICINAL PRODUCTS AND OTHER FORMS OF INTERACTION

Effects of other medicinal products on sitagliptin

The risk for clinically meaningful interactions by co-administered medicinal products is low.

The primary enzyme responsible for the limited metabolism of sitagliptin is CYP3A4, with contribution from CYP2C8. In patients with normal renal function, metabolism, including via CYP3A4, plays only a small role in the clearance of sitagliptin. Metabolism may play a more significant role in the elimination of sitagliptin in the setting of severe renal impairment or end-stage renal disease (ESRD). For this reason, it is possible that potent CYP3A4 inhibitors (i.e., ketoconazole, itraconazole, ritonavir, clarithromycin) could alter the pharmacokinetics of sitagliptin in patients with severe renal impairment or ESRD. The effect of potent CYP3A4 inhibitors in the setting of renal impairment has not been assessed.

Sitagliptin is a substrate for p-glycoprotein and organic anion transporter-3 (OAT3). OAT3 mediated transport of sitagliptin was inhibited in vitro by probenecid, although the risk of clinically meaningful interactions is considered to be low. Concomitant administration of OAT3 inhibitors has not been evaluated.

Metformin: Co-administration of multiple twice-daily doses of 1,000 mg metformin with 50 mg sitagliptin did not meaningfully alter the pharmacokinetics of sitagliptin in patients with type 2 diabetes.

Ciclosporin: Not considered to be clinically meaningful. The renal clearance of sitagliptin was not meaningfully altered. Therefore, meaningful interactions would not be expected with other p glycoprotein inhibitors.

Effects of sitagliptin on other medicinal products

Digoxin: Sitagliptin had a small effect on plasma digoxin concentrations. No dose adjustment of digoxin is recommended. However, patients at risk of digoxin toxicity should be monitored for this when sitagliptin and digoxin are administered concomitantly.

Sitagliptin does not inhibit nor induce CYP450 isoenzymes. Sitagliptin did not meaningfully alter the pharmacokinetics of metformin, glyburide, simvastatin, rosiglitazone, warfarin, or oral contraceptives, providing evidence of a low propensity for causing interactions with substrates of CYP3A4, CYP2C8, CYP2C9, and organic cationic transporter (OCT). Sitagliptin may be a mild inhibitor of p-glycoprotein.

4.6 FERTILITY, PREGNANCY AND LACTATION

Pregnancy
There are no adequate data from the use of sitagliptin in pregnant women. Studies in animals have shown reproductive toxicity at high doses. The potential risk for humans is unknown. Due to lack of human data, Sitagliptin should not be used during pregnancy.

Breast-feeding
It is unknown whether sitagliptin is excreted in human breast milk. Animal studies have shown excretion of sitagliptin in breast milk. Sitagliptin should not be used during breast-feeding.

Fertility
Animal data do not suggest an effect of treatment with sitagliptin on male and female fertility. Human data are lacking.

4.7 EFFECTS ON ABILITY TO DRIVE AND USE MACHINES

DIASIT has no or negligible influence on the ability to drive and use machines. However, when driving or using machines, it should be taken into account that dizziness and somnolence have been reported.

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In addition, patients should be alerted to the risk of hypoglycaemia when DIASIT is used in combination with a sulphonylurea or with insulin.

4.8 UNDESIRABLE EFFFECTS

Summary of the safety profile

Serious adverse reactions including pancreatitis and hypersensitivity reactions have been reported. Hypoglycaemia has been reported in combination with sulphonylurea and insulin.

Tabulated list of adverse reactions

Adverse reactions are listed below (Table 1) by system organ class and frequency. Frequencies are defined as: very common; common; uncommon; rare; very rare and not known (cannot be estimated from the available data).

Table 1: The frequency of adverse reactions identified from placebo-controlled clinical studies of sitagliptin monotherapy and post-marketing experience.

Adverse reaction	Frequency of adverse reaction
Blood and lymphatic system disorders	
Thrombocytopenia	Rare
Immune system disorders	
Hypersensitivity reactions include anaphylactic responses*,†	Frequency not known
Metabolism and nutrition disorders	
Hypoglycaemia†	Common
Nervous system disorders	
Headache	Common
Dizziness	Uncommon
Respiratory, thoracic and mediastinal disorders	
Interstitial lung disease*	Frequency not known
Gastrointestinal disorders	
Constipation	Uncommon
Vomiting*	Frequency not known
Acute pancreatitis*,†	Frequency not known
Fatal and non-fatal haemorrhagic and necrotizing pancreatitis*,†	Frequency not known
Skin and subcutaneous tissue disorders	
Pruritus*	Uncommon
Angioedema*,†	Frequency not known
Rash*,†	Frequency not known
Urticaria*,†	Frequency not known
Cutaneous vasculitis*,†	Frequency not known
Exfoliative skin conditions including Stevens-Johnson*,†	Frequency not known
Bullous pemphigoid*	Frequency not known
Musculoskeletal and connective tissue disorders	
Arthralgia*	Frequency not known
Myalgia*	Frequency not known
Back pain*	Frequency not known
Arthropathy*	Frequency not known
Renal and urinary disorders	
Impaired renal function*	Frequency not known
Acute renal failure*	Frequency not known

* Adverse reactions were identified through post-marketing

† See section: Special Warnings and Precautions for Use

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Description of selected adverse reactions

In addition to the drug-related adverse experiences described above, adverse experiences reported regardless of causal relationship to medication and occurring in at least 5 % and more commonly in patients treated with

sitagliptin included upper respiratory tract infection and nasopharyngitis. Additional adverse experiences reported regardless of causal relationship to medication that occurred more frequently in patients treated with sitagliptin included osteoarthritis and pain in extremity.

Some adverse reactions were observed more frequently in combination use of sitagliptin with other antidiabetic medicinal products than in studies of sitagliptin monotherapy. These included hypoglycaemia (frequency very common with the combination of sulphonylurea and metformin), influenza (common with insulin (with or without metformin)), nausea and vomiting (common with metformin), flatulence (common with metformin or pioglitazone), constipation (common with the combination of sulphonylurea and metformin), peripheral oedema (common with pioglitazone or the combination of pioglitazone and metformin), somnolence and diarrhoea (uncommon with metformin), and dry mouth (uncommon with insulin (with or without metformin)).

Paediatric population

In paediatric patients with type 2 diabetes mellitus aged 10 to17 years, the profile of adverse reactions was comparable to that observed in adults.

4.9 OVERDOSE

There is no experience with doses above 800 mg. There were no dose-related clinical adverse reactions observed with sitagliptin with doses of up to 600 mg per day for periods of up to 10 days and 400 mg per day for periods of up to 28 days.

In the event of an overdose, it is reasonable to employ the usual supportive measures, e.g., remove unabsorbed material from the gastrointestinal tract, employ clinical monitoring (including obtaining an electrocardiogram), and institute supportive therapy if required.

Sitagliptin is modestly dialysable. Approximately 13.5 % of the dose was removed over a 3- to 4-hour haemodialysis session. Prolonged haemodialysis may be considered if clinically appropriate. It is not known if sitagliptin is dialysable by peritoneal dialysis.

5.0 PHARMACOLOGICAL PROPERTIES
5.1 Pharmacodynamic properties

Pharmacotherapeutic group: Drugs used in diabetes, Dipeptidyl peptidase 4 (DPP-4) inhibitors, ATC code: A10BH01.

Mechanism of action

DIASIT is a member of a class of oral anti-hyperglycaemic agents called dipeptidyl peptidase 4 (DPP-4) inhibitors. The improvement in glycaemic control observed with this medicinal product may be mediated by enhancing the levels of active incretin hormones. Incretin hormones, including glucagon-like peptide-1 (GLP-1) and glucose-dependent insulinotropic polypeptide (GIP), are released by the intestine throughout the day, and levels are increased in response to a meal. The incretins are part of an endogenous system involved in the physiologic regulation of glucose homeostasis.

When blood glucose concentrations are normal or elevated, GLP-1 and GIP increase insulin synthesis and release from pancreatic beta cells by intracellular signalling pathways involving cyclic AMP. Treatment with GLP-1 or with DPP-4 inhibitors has been demonstrated to improve beta cell responsiveness to glucose and stimulate insulin biosynthesis and release. With higher insulin levels, tissue glucose uptake is enhanced. In addition, GLP-1 lowers glucagon secretion from pancreatic alpha cells. Decreased glucagon concentrations, along with higher insulin levels, lead to reduced hepatic glucose production, resulting in a decrease in blood

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glucose levels. The effects of GLP-1 and GIP are glucose-dependent such that when blood glucose concentrations are low, stimulation of insulin release and suppression of glucagon secretion by GLP-1 are not observed. For both GLP-1 and GIP, stimulation of insulin release is enhanced as glucose rises above normal concentrations. Further, GLP-1 does not impair the normal glucagon response to hypoglycaemia. The activity of GLP-1 and GIP is limited by the DPP-4 enzyme, which rapidly hydrolyzes the incretin hormones to produce inactive products. Sitagliptin prevents the hydrolysis of incretin hormones by DPP-4, thereby increasing plasma concentrations of the active forms of GLP-1 and GIP. By enhancing active incretin levels, sitagliptin increases insulin release and decreases glucagon levels in a glucose-dependent manner.

In type 2 diabetes with hyperglycaemia, these changes in insulin and glucagon levels lead to lower haemoglobin A_{1c} (HbA_{1c}) and lower fasting and postprandial glucose concentrations. The glucose-dependent mechanism of sitagliptin is distinct from the mechanism of sulphonylureas, which increase insulin secretion even when glucose levels are low and can lead to hypoglycaemia in type 2 diabetes. Sitagliptin is a potent and highly selective inhibitor of the enzyme DPP-4 and does not inhibit the closely-related enzymes DPP-8 or DPP-9 at therapeutic concentrations.

Sitagliptin alone increased active GLP-1 concentrations, whereas metformin alone increased active and total GLP-1 concentrations to similar extents. Co-administration of sitagliptin and metformin had an additive effect on active GLP-1 concentrations. Sitagliptin, but not metformin, increased active GIP concentrations.

5.2 Pharmacokinetic properties

The pharmacokinetics of sitagliptin have been extensively characterized in healthy subjects and patients with type 2 diabetes. After oral administration of a 100-mg dose to healthy subjects, sitagliptin was rapidly absorbed, with peak plasma concentrations (median T_{max}) occurring 1 to 4 hours post-dose. Plasma AUC of sitagliptin increased in a dose-proportional manner. Following a single oral 100-mg dose to healthy volunteers, mean plasma AUC of sitagliptin was 8.52 µM•hr, C_{max} was 950 nM, and apparent terminal half-life ($t_{1/2}$) was 12.4 hours.

Plasma AUC of sitagliptin increased approximately 14 % following 100-mg doses at steady-state compared to the first dose. The intra-subject and inter-subject coefficients of variation for sitagliptin AUC were small (5.8 % and 15.1 %). The pharmacokinetics of sitagliptin were generally similar in healthy subjects and in patients with type 2 diabetes.

Absorption

The absolute bioavailability of sitagliptin is approximately 87 %. Since co-administration of a high-fat meal with sitagliptin had no effect on the pharmacokinetics, sitagliptin may be administered with or without food.

Distribution

The mean volume of distribution at steady state following a single 100-mg intravenous dose of sitagliptin is approximately 198 litres. The fraction of sitagliptin reversibly bound to plasma proteins is low (38 %).

Metabolism

Sitagliptin is primarily eliminated unchanged in urine, and metabolism is a minor pathway. Approximately 79 % of sitagliptin is excreted unchanged in the urine.

Following a [^{14}C] sitagliptin oral dose, approximately 16 % of the radioactivity was excreted as metabolites of sitagliptin. Six metabolites were detected at trace levels and are not expected to contribute to the plasma DPP-4 inhibitory activity of sitagliptin. In vitro studies indicated that the primary enzyme responsible for the limited metabolism of sitagliptin was CYP3A4, with contribution from CYP2C8.

Elimination

Following administration of an oral [^{14}C] sitagliptin dose to healthy subjects, approximately 100 % of the administered radioactivity was eliminated in feces (13 %) or urine (87 %) within one week of dosing. The

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apparent terminal t1/2 following a 100-mg oral dose of sitagliptin was approximately 12.4 hours and renal clearance was approximately 350 mL/min.

Elimination of sitagliptin occurs primarily via renal excretion and involves active tubular secretion. Sitagliptin is a substrate for human organic anion transporter-3 (hOAT-3), which may be involved in the renal elimination of sitagliptin. The clinical relevance of hOAT-3 in sitagliptin transport has not been established. Sitagliptin is also a substrate of p-glycoprotein, which may also be involved in mediating the renal elimination of sitagliptin. However, cyclosporine, a p-glycoprotein inhibitor, did not reduce the renal clearance of sitagliptin.

Characteristics in patients

Renal impairment

A single-dose, open-label study was conducted to evaluate the pharmacokinetics of sitagliptin (50-mg dose) in patients with varying degrees of chronic renal impairment compared to normal healthy control subjects. The study included patients with mild, moderate, and severe renal impairment, as well as patients with ESRD on hemodialysis. In addition, the effects of renal impairment on sitagliptin pharmacokinetics in patients with type 2 diabetes and mild, moderate or severe renal impairment (including ESRD) were assessed using population pharmacokinetic analyses.

Compared to normal healthy control subjects, plasma AUC of sitagliptin was increased by approximately 1.2-fold and 1.6-fold in patients with mild renal impairment (eGFR $\geq$ 60 mL/min/1.73 m^2 to < 90 mL/min/1.73 m^2) and patients with moderate renal impairment (eGFR $\geq$ 45 mL/min/1.73 m^2 to < 60 mL/min/1.73 m^2), respectively. Because increases of this magnitude are not clinically relevant, dosage adjustment in these patients is not necessary.

Plasma AUC of sitagliptin was increased approximately 2-fold in patients with moderate renal impairment (eGFR $\geq$ 30 mL/min/1.73 m^2 to < 45 mL/min/1.73m^2), and approximately 4-fold in patients with severe renal impairment (eGFR < 30 mL/min/1.73 m^2), including patients with ESRD on hemodialysis. Sitagliptin was modestly removed by hemodialysis (13.5 % over a 3 to 4 hour hemodialysis session starting 4 hours postdose). To achieve plasma concentrations of sitagliptin similar to those in patients with normal renal function, lower dosages are recommended in patients with eGFR <45 mL/min/1.73 m^2.

Hepatic impairment

In patients with moderate hepatic impairment (Child-Pugh score 7 to 9), mean AUC and C_{max} of sitagliptin increased approximately 21 % and 13 %, respectively, compared to healthy matched controls following administration of a single 100-mg dose of sitagliptin. These differences are not considered to be clinically meaningful. No dosage adjustment for sitagliptin is necessary for patients with mild or moderate hepatic impairment.

There is no clinical experience in patients with severe hepatic impairment (Child-Pugh score >9). However, because sitagliptin is primarily renally eliminated, severe hepatic impairment is not expected to affect the pharmacokinetics of sitagliptin.

Elderly

No dose adjustment is required based on age. Age did not have a clinically meaningful impact on the pharmacokinetics of sitagliptin based on a population pharmacokinetic analysis of Phase I and Phase II data. Elderly subjects (65 to 80 years) had approximately 19 % higher plasma concentrations of sitagliptin compared to younger subjects.

Paediatric population

The pharmacokinetics of sitagliptin (single dose of 50 mg, 100 mg or 200 mg) were investigated in paediatric patients (10 to 17 years of age) with type 2 diabetes. In this population, the dose-adjusted AUC of sitagliptin in plasma was approximately 18 % lower compared to adult patients with type 2 diabetes for a 100 mg dose. This is not considered to be a clinically meaningful difference based on the flat PK/PD relationship between the dose of 50 mg and 100 mg in adults.

No studies with sitagliptin have been performed in pediatric patients < 10 years of age.

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Gender

No dosage adjustment is necessary based on gender. Gender had no clinically meaningful effect on the pharmacokinetics of sitagliptin based on a composite analysis of Phase I pharmacokinetic data and on a population pharmacokinetic analysis of Phase I and Phase II data.

Race

No dosage adjustment is necessary based on race. Race had no clinically meaningful effect on the pharmacokinetics of sitagliptin based on a composite analysis of Phase I pharmacokinetic data and on a population pharmacokinetic analysis of Phase I and Phase II data, including subjects of white, Hispanic, black, Asian, and other racial groups.

Body Mass Index (BMI)

No dosage adjustment is necessary based on BMI. Body mass index had no clinically meaningful effect on the pharmacokinetics of sitagliptin based on a composite analysis of Phase I pharmacokinetic data and on a population pharmacokinetic analysis of Phase I and Phase II data.

Type 2 Diabetes

The pharmacokinetics of sitagliptin in patients with type 2 diabetes are generally similar to those in healthy subjects.

6. PHARMACEUTICAL PARTICULARS

6.1 List of excipients

Excipients for core tablet:

Microcrystalline cellulose, Calcium hydrogen phosphate, Croscarmellose sodium, Sodium stearyl fumarate, Magnesium stearate.

Film coating:

DIASIT 25: Opadry II Complete Film Coating System 85F540307 Pink

DIASIT 50: Opadry II Complete Film Coating System 85F570116 Light Beige

DIASIT 100: Opadry II Complete Film Coating System 85F570063 Beige

6.2 Incompatibilities

None

6.3 Shelf Life

24 months (please refer to the outer box for expiry date)

6.4 Special precautions for storage

Store below 30°C.

6.5 Nature and Contents of Container

DIASIT 25, 50 and 100 are available in boxes of 30 tablets, 3 blisters (white opaque PVC/PVdC – printed alu blisters) of 10 tablets each are packed in an outer box with an information leaflet.

6.6 Special precautions for disposal and other handling

Any unused medicinal product or waste material should be disposed of in accordance with local requirements.

7. MANUFACTURER AND PRODUCT REGISTRATION HOLDER

Manufacturer

Aurobindo Pharma Limited, Unit XV
Plot. No. 17 A, E Bonangi Village,
Parawada Mandal, Anakapalli District,
Anakapalli, 531021 India

Product Registration Holder

Synerrv Sdn. Bhd.
SO-29-2, Menara 1, KL Eco City, Jalan Bangsar,
Kg. Haji Abdullah Hukum,
59200 Kuala Lumpur, Malaysia

8. LEAFLET REVISION DATE

08 December 2023

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